

Contraband and Contraband Searches

Policy & Procedure SGHC020587

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Purpose

To provide a safe environment for all patients, staff, and visitors, and to give direction to all hospital staff on what is identified as contraband and how it will be managed when discovered.

Policy

- A. All items, commercial or otherwise, which reasonably may be used to aid in an assault, escape, self-injury or disrupt the milieu, shall be confiscated, and secured or disposed of.
- B. Patients' rights, dignity, privacy, and safety shall be protected during necessary search procedures.
- C. All searches shall be reasonable and conducted at the lowest level of invasiveness that is consistent with health and safety.
- D. Physicians who wish to write orders allowing items typically considered contraband are advised to consult with the Chief Medical Officer and to inform the Security Department.

Definitions

1. **Contraband**—Any item that is prohibited because it is deemed inappropriate for storage/usage in the hospital, or within a specific area of the hospital or potentially disruptive to the milieu because it reasonably may be used to aid in an assault, escape, self-injury, or intimidation, within the context of the individual patient and the patient population on the unit.
2. **Chain of Custody**—The possession, movement, and location of physical evidence or contraband from the time it is obtained, until the time it is presented in court, stored, or turned over to MDH Police. This includes the documentation of the possession, movement, and location of the item at each point.
3. **Search**— To examine a person and/or the possessions of a person in order to rule out contraband or to locate something lost or concealed. Some

searches are considered routine (see definition of "Routine Search"), while other searches are done under special circumstances and considered to be special and non routine. Also, certain searches may be done on an individual basis, while other searches involve groups of individuals. The types of search procedures authorized for use within this facility, in ascending order of invasiveness are environmental search; possessions search; external body search; frisk; strip search; and body cavity search.

4. **Routine Search**— Routine searches are those searches that are authorized under this policy and procedure to be routinely performed under the following circumstances:
 - a. A search of a visitor and/or the visitor's possessions at the time of arrival for a visit, or at any time the visitor leaves the building or secured area and returns.
 - b. A search of a patient and/or the patient's possessions upon admission or transfer; or upon return from a leave of absence; or upon return from elopement/escape; or upon return from unescorted grounds privileges, work assignment, or off-grounds trip; or as part of a routine unannounced search.

Only environmental, possessions, and external body searches are done as routine searches.

5. **Special Non-Routine Search**— A special non-routine search is a search that is authorized under this policy and procedure to be performed under special circumstances for reasonable cause. A special, non-routine search may be limited to an environmental search, a possessions search, an external body search, and/or a frisk search; or, as circumstances require, special searches may involve more

invasive examinations such as strip search or body cavity search. Non-routine searches may be performed on a special basis any time a Registered Nurse, a Security Officer, a Police Officer, or a Physician reasonably suspects that contraband is in the possession or control of a patient.

6. **Environmental Search**—An environmental search is a contraband search method that involves the search of living and treatment spaces. Environmental searches are conducted by Security and/or clinical staff in order to ensure compliance with specific hospital policies that pertain to infection control, personal property, patient safety and the environment of care. Environmental searches include searches of physical spaces, furniture, and equipment. Environmental searches should not be confused with inspections conducted on the environment of care which are done to identify and address maintenance and housekeeping issues.
7. **Possessions Search**—A Possessions Search is a contraband search method that involves the search of personal possessions and packages. This form of search requires inspection of items belonging to the patient/visitor and/or in his/her possession, to include items on the unit, in storage or brought into the hospital. Examples of possessions that may be searched include, but are not limited to, books and magazines, packages, purses, wallets, containers, backpacks, bags, stored clothing, boxes, cosmetics, and similar items.
8. **External Body Search**— An external body search is a contraband search method that involves visual inspection of the clothed person, and may include the removal of the person's hat, shoes, and socks, as well as a request for the individual to turn his or her pockets inside out in order to assure that they are free of contraband. An external body search may include using a metal detector (walk through or handheld) to check for the presence of metallic contraband.
9. **Frisk**— A frisk is a search method that involves passing/patting hands over the outer clothing of a person in order to check for contraband. Frisking is a search method that is considered to be more invasive than a possessions search, an environmental search, and an external body search. Frisk searches of patients and visitors may be done by the Security Officers, as indicated, but are not done involuntarily with patients without the order of a physician. Unlike environmental, possessions, and external body searches, frisk searches must be done by the Security Officers and are not done by clinical staff. A visitor who refuses to submit to being frisked in accordance with this policy may be asked to leave or may not be permitted to visit.
10. **Strip Search**— A strip search is an individual contraband search method that requires removal of clothing that exposes the breasts, buttocks, and/or genitals. Strip searches, by their nature, are more invasive than environmental, possessions, external body, and frisk searches, and therefore require a higher degree of suspicion and cause. A strip search can never be done routinely and requires an order by a physician. A strip search would never be done on a visitor.
11. **Body Cavity Search**— A body cavity search is an individual contraband search method that involves the visual or manual inspection of a subject's gluteal cleft, rectum, and/or vagina. For the purposes of this policy and procedure, a medical or dental examination is not considered to be a body cavity search. Similarly, for the purposes of this policy and procedure, inspection of the oral cavity is not considered to be a body cavity search. Oral cavity inspection may be done routinely as part of medication administration. Body Cavity Searches, by their nature, are more invasive than environmental, possessions, external body, frisk, and strip searches, and therefore require an even greater degree of suspicion and cause. A body cavity search can never be done routinely and requires an

order by a physician and is referred by clinical staff. A body cavity search is never done on a visitor.

12. **Metal Detector**—A device or instrument used to detect metal objects carried by a person. Such device may be a walkthrough or handheld.

I. Contraband Items:

1. **Weapons:** Any item which could reasonably be used as weapon. The hospital clinical staff, in consultation with the Security Department, will make a determination as to whether an object may reasonably be considered a weapon.

The following items shall be considered weapons in all instances:

- firearms of any type loaded or unloaded,
- knives, and other sharp cutting instruments,
- clubs, and other bludgeon devices,
- mace, and other chemical products.

2. **Sharp Objects:** This includes any object, which, in either its original or altered state, could be used to inflict injury. This shall include but not be limited to:

- any kind of glass object, including but not limited to glass mirrors, decorative objects, bottles, or other glass containers,
- wood (that might be whittled or splintered into a weapon),
- scissors,
- any other sharp object, or any other item that could reasonably be made into a sharp object, such as compact disks (CD's) and digital video disks (DVD's), credit or debit cards.
- any object containing or having as a part of it, metal, including clothes hangers,
- pens, and pencils other than "shorty" and,
- certain toothbrushes that are constructed of hard plastics.

3. **Ligatures:** Objects that can be used for asphyxiation such as rope, electric cords, headphones with cords, eyeglass chains/cords, plastic bags, scarves, neckties, belts, drawstrings, and shoelaces.
4. **Drugs and Drug Products:** Unless ordered by a physician privileged to practice at Spring Grove Hospital Center, no patient may maintain any drug or drug product in his/her personal possession. This prohibition shall include prescription, non-prescription medication, and herbal products. Illicit drugs and spirited beverages are always considered contraband.
5. **Personal Hygiene Products:** Items in glass or aerosol containers such as deodorants, shaving cream or perfumes over a 30% alcohol level and items containing ethyl alcohol. Items that are operated by electricity such as electric razors, curlers, and hair dryers. Exceptions may be made by physician order to allow such items to be held in nursing custody and used by a patient for brief periods. Disposable razors are dispensed to patients for shaving purposes by nursing staff for 15-minute time periods. (See Razor Use Nursing Policy).
6. **Flammable Substances:** To include matches, cigarette lighters, flammable liquids, gases, and chemicals.
7. **Tobacco Products:** All tobacco products are contraband at Spring Grove Hospital Center. These include, but are not limited to cigars, cigarettes, snuff, chewing tobacco, and e-cigarettes/vapor cigarettes.
8. **Entertainment Appliances:** Items that are operated by "house" electric current or an electrical cord for use. Also included are items requiring the use of batteries. This includes items such as radios, DVD players, or television sets. (Exceptions are personal radio headsets with rubber antennae).

9. **Cameras, Video and Audio Recording Devices:** In order to assure patient confidentiality, and to preserve the therapeutic milieu, no types of photographic or recording devices are permitted unless authorized by policy ("Photographing, Videotaping and/or Audiotaping Patient").
10. **Cellular Phones and Pagers/Beepers, Smart Phone, Laptop Computers, and Tablet Computers:** To assure privacy and to provide a therapeutic environment, cell phones, pagers/beepers, computers, and tablet computers are contraband for patients and visitors. Exceptions are made for certain visitors who require such items as necessary business equipment.
11. **Items that Could Cause Blunt Trauma:** All steel toed shoes and boots.
12. **Poisons:** Poisons, poisonous materials, and toxic substances.
13. **Pornography:** Pornographic materials (e.g., magazines, books, pictures).
14. **Other Items:** An item may be considered to be contraband if the physician, and/or other clinical members of the treatment team, or Police/Security Officer reasonably considers the item to pose a threat to the preservation of the milieu or the safety of patients or staff, within the context of the individual patient's condition and the patient population on a unit at any given time. Examples of items that may be considered contraband under certain circumstances include large amounts of cash or other negotiable valuables, heavy jewelry, such as large rings that could potentially be used to inflict injury, and batteries that could be swallowed or used to start a fire under certain circumstances. Patient personal belongings can be considered contraband if the treatment team reasonably determines that a patient is storing an abnormal amount of items in the room and that such items pose a health, access, or safety risk. If patients are not properly storing personal

belongings, and if the patient is compromising the rights, privacy, or safety of residents and staff by the improper use of storage of personal items, then these items shall be stored in a secure area off of the unit. Some patient items are kept in the nurse's station for limited use such as headphones and cosmetics.

Definitions

II. Routine Searches of Visitors and Packages Brought in by Visitors:

1. All units and building lobbies will have signage- "Notice to Patients and Visitors"- prominently displayed for viewing that summarizes our contraband policy and includes many items considered to be contraband. The visitor shall be shown the current contraband list upon request. The list shall be available on all patient units. Included in the patient's admission information will be a "Patient Safety Information for Visitors" letter.
2. **Packages and parcels brought to patients by visitors:** All packages and parcels, which are brought to patients by visitors are to be searched for contraband by a security officer or by a nursing staff member before the package or parcel is given to the patient. The visitor must always be given the option of being present while the packages and parcels brought in by the visitor are being searched.
3. Any visitor who knowingly attempts to bring contraband into the Hospital risks being arrested and/or facing criminal charges.
4. Some visitors may unknowingly or inadvertently bring in contraband items which are found during a routine search. If this occurs, the visitor will be educated by the Security Officer or nursing staff on the Hospital policy.
5. When it is discovered that a visitor is knowingly attempting to smuggle in contraband, or if a patient is found in possession of contraband after a

specific person has visited, that visitor will be placed on an "Enhanced Security List". This means that when this visitor arrives, they will be frisked by a Security Officer prior to being allowed to visit. In some cases, visitors may not be allowed to return until they meet with a member of the Security Department. Their future visitations may have to be scheduled through and or coordinated with the Security Department.

6. **Visitor Searches:** All visitors are required to leave outerwear, such as coats, hats, jackets, and similar garments, in the lobby storage. Contraband electronic items, such as cell phones and laptop computers are to be similarly secured. Certain exceptions apply (See Section I- 9 and 10). Visitor searches are conducted as follows:

- a. **Dayhoff and White Buildings:** Visitors to the Dayhoff and White Buildings are required to pass through a metal detector and undergo a search by a Security Officer of the possessions the visitor intends to carry with them onto the locked unit. The types of searches that may be conducted include possessions searches, external body searches, and frisk searches. The Security Officer is authorized to secure any contraband item that may be discovered. Questions as to whether a particular item should be considered contraband should be addressed to the Nurse Manager on the unit where the patient who is to be visited is assigned. The Nurse Manager and the Hospital Security Officer have the authority to deny access and ask the visitor to leave if the visitor unreasonably declines all or any portion of the

search. In the absence of a Security Officer, nursing staff on the unit may conduct the search with the aid of a hand-held metal detector but must call for a Security Officer if a frisk search is considered to be necessary.

- b. **Continued Care Units, Smith, and Tawes Building:** Visitors will be checked by Security Officers upon arrival to the lobby by the handheld wand on the unit. However, if the visitor is unknown to the staff, or in the event that there is cause for suspicion (for example, the patient who is to be visited has been found to be in possession of contraband from an unknown source), a Security Officer or a member of the nursing staff may conduct a search of the visitor, with the aid of a hand-held metal detector. The Security Officer or nursing staff member is authorized to secure any contraband item that may be discovered during the search of the visitor. The Security Officer and nursing staff, under the guidance of the Nurse Manager or the charge nurse have the authority to deny access if there is a reasonable suspicion of the presence of contraband and the visitor declines to be searched.

III. Routine Patient Searches:

1. **Environmental, Possession, and External Body:** All patients upon entering the inpatient buildings or units under the following circumstances must routinely undergo contraband searches: Admission or transfer; upon return from a leave of absence; upon return from elopement/escape; upon return from unescorted grounds privileges, or work assignment, off-

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grounds trip; as part of an unannounced search. Any package, parcel, purse, backpack, bag, suitcase, or similar container that the patient may be carrying must be manually inspected by a Security Officer or by a member of the unit nursing staff. The packages and parcels may be scanned with a metal detector. Patients have the right to be present while their belongings are searched, and any declination of this right must be documented in the form of a progress note in the patient's medical record. Patients must also undergo an external body search, and a frisk search may be conducted as indicated. Only Hospital Security Officers and Police are also authorized to conduct a frisk, as indicated, under the provisions of this policy and procedure.

2. Routine Unannounced Contraband Searches:

Because of Spring Grove Hospital Center's fiduciary obligation to its patients, staff, and visitors – and because contraband items are routinely discovered in the possession of patients of the Center, routine unannounced searches of the entire unit may be conducted periodically. Routine unannounced searches are conducted only under the approval and guidance of the Psychiatrist, Nurse Manager or Charge Nurse of each unit. A search of the environment, of possessions, and of the person are permitted, under the provisions of this policy and procedure. Such routine searches are generally carried out by designated Security Officers, and/or nursing staff. Upon completion of unit searches, the Charge Nurse shall notify the Psychiatrist, the Nurse Manager, and, as may be required, the Nursing Administration Office and the MDH Police of the results of the search. The Charge Nurse shall document on the 24-hour Report that the search had been conducted and the results.

Examples of specific areas that are to be searched during routine unannounced searches of the environment include:

- a. **Environmental Search:** A search of the environment include under mattresses, in pillowcases, on the underside of furniture, in books and magazines, in light fixtures, under toilets and sinks, in tissue boxes, toilet paper rolls, etc.
- b. **Possessions Search:** A search of the patient's possessions, packages, bags, boxes, suitcases, clothing, and similar items may be performed, as reasonable, by a Security Officer, or clinical staff during any routine search.
- c. **External Body Search:** An external body search for contraband may be performed, as reasonable, by Security Officer, and/or clinical staff during any routine search.
- d. **Frisk:** A frisk for contraband may be performed, as reasonable by Security Officers with a physician order. Except as noted below, a frisk search for contraband is to be performed by a person of the same gender as the individual who is being searched.

IV. Special Non-Routine Searches:

Special unannounced searches may be conducted of an entire unit, a portion thereof, or a common area used by patients (gymnasium, activity room, dining room, etc.) for arising cause, and may be accomplished under the authority of a Physician, the Nurse Manager or Charge Nurse.

1. During environmental searches of a unit or portion of a unit, staff should search clothes, furniture, drawers, wardrobes, closets, and bathrooms. These are different from individual searches in

that they may involve more than one person, simultaneously, and are not part of a system of routine unannounced.

2. **Strip Searches:** A strip search may only be done upon the order of a physician, and may only be performed by a physician, a physician's assistant, or a Registered Nurse, in the presence of at least one other clinical staff member. Except as noted, in "Special Exceptions"- #4, strip searches may only be performed by individuals who are of the same gender as the individual who is being searched. No more than one person may be strip searched in the same room at the same time. A strip search is performed only when there is reasonable cause to believe that a search of this type is reasonably necessary and justified in order to find a contraband item that may be hidden on the individual's person. A history of sexual trauma must always be considered in such cases, in relation to the relative risk. Strip searches must always be performed in such a manner as to permit the patient as much privacy, dignity, and comfort as possible.

3. **Body Cavity Search:** A body cavity search may only be done upon the order of a physician and may only be performed by a Physician or a Physician's Assistant, in the presence of a Registered Nurse. The Physician, the Physician's Assistant, or the Registered Nurse must be of the same gender as the patient, except as noted in "Special Exceptions"- #4. A cavity search is performed only when there is reasonable cause to believe that a search of this type is reasonably necessary and justified in order to find a contraband item that may be hidden in one of the individual's body cavities. A history of sexual trauma must always be considered, and such searches must always be performed in such a manner as to permit the patient as much privacy, dignity, and comfort as possible.

4. **Special Exceptions:** Special exceptions may be made, as appropriate. For example, in the case of pre-surgical transgendered patient, the patient's gender of identity must be considered to be the patient's gender and so, in consultation with the Chief Medical Officer or Chief Medical Officer's designee (who is usually the On-Duty physician), the search is to be performed by a person of the same gender as the patient's identity, unless the patient requests otherwise. Either the physician or RN will be of the same gender as the patient. Similarly, the victim of same-sex rape or other sexual trauma may request that they be examined by a member of the opposite sex. Such requests should be directed by staff to the Chief Medical Officer or the Chief Medical Officer's designee for a determination.

V. General Guidelines for all type Searches:

Regardless of the type and reason for the search, the search shall always be conducted in a dignified and professional manner and the following considerations are to be followed:

1. **Reasonableness:** The degree of invasiveness of the search must be in reasonable proportion to the relative risk associated with the suspected contraband item, and further balanced by such factors as the level and nature of the suspicion, and the risk of further traumatizing a victim of physical or sexual abuse. Targeted non-routine searches must be based upon personal observation or other reliable information that indicates that the subject possesses, or reasonably may possess contraband. Only a physician or RN may make the determination as to whether such searches are to occur.
2. **Profiling Prohibited:** Searches and search method must never be based upon racial, ethnic, gender, age, or other forms of profiling.
3. **Explanation to Patient:** Patients are to be notified by nursing staff during the patient's orientation to the unit that they will be subject to random unannounced searches

of their possessions. Further, and at the discretion of the Psychiatrist, patients may be reminded by staff, for example in community meetings, that unannounced searches will be conducted and that they can voluntarily turn in contraband to nursing staff. At the time of the search of an entire unit or targeted area within the unit, either individually or in a community meeting, the affected patients are to be informed of the search and its purpose. If possible, each room shall be searched with the occupants present while all other patients remain in the dayroom.

4. **Privacy and Dignity:** Searches that require the subject to disrobe (except for shoes, socks, gloves, coats or other outerwear, sweaters, sweatshirts, hats, and similar garments) must be performed in the presence of at least two clinical staff members, who are of the same sex as the person being searched. (Exceptions noted in Special Exceptions – IV, #4). A Security Officer may also be present inside the room of the same gender as the patient. Otherwise, they should wait immediately outside the door in case emergency assistance is needed. Searches shall always be conducted in a dignified and professional manner.
5. **Documentation Requirements:** The Charge Nurse or designee is responsible for documenting on a progress note the following: date, time, and the justification for any physical search of the patient or the patient's room; the findings and disposition of any contraband items; any subsequent actions taken with the patient; and names of staff members involved. The Charge Nurse or designee shall document observations of the patient's condition and the rationale for implementation of this procedure in a progress note before the end of the shift. A Management Variance Report (MVR) must be completed regarding all contraband items that are found and must note the manner of disposition of the contraband items.

VI. Obtaining/Confiscating Contraband:

1. The individual's name shall be attached to the property, along with the date that the property was obtained or confiscated. The signature of the hospital staff person who found the contraband shall also be included. A chain of custody will be established and maintained.
2. The person from whom the contraband item is being taken must be notified that the contraband item(s) has or have been found and confiscated.
3. If a patient is involved:
 - a. The patient's off-unit privileges may be temporarily suspended, at least until the source of the contraband is determined.
 - b. The matter is referred to the unit treatment team for assessment and intervention, as required.
 - c. The incident shall be documented in the patient's progress notes and 24-hour Shift Report.
 - d. The Psychiatrist, the Nurse Manager, the Nursing Administration Office, are to be notified if any contraband item is found and confiscated during a search.
 - e. The nursing staff will complete and submit "SGHC Personal Effects Inventory Form", documenting who the property was taken from, describing the property, signing, and dating the lower portion of the form. If appropriate, the patient's name should be affixed to the item(s). The employee will have the person that receives the property from them sign the form as well. A chain of custody will be established and maintained.
 - f. The Security Officer will take possession of the property and place it in an evidence envelope and drop the item in the property drop box. If an officer is not available, the individual that is in possession of the property may drop the item in the property drop box located in the Dayhoff Lobby or the Preston Building. Make sure it is marked as evidence.

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VII. Disposition/Disposal of Contraband, Additional Requirements:

1. If Security takes possession of contraband, a supervisor can determine if the contraband needs to be passed on MDH Police.
 - a. Contraband items that are to be returned to the patient upon discharge shall be placed in locked storage during his/her stay at the hospital. Depending on the type of item, it can be stored on the unit.
 - b. For contraband items confiscated from an adolescent patient, the patient's parent or legal guardian(s) is/are to be notified if clinically indicated. The notification of the family, or the reason for not notifying the family is documented on a progress note by a Registered Nurse.
 - c. **Tobacco and Alcohol:** If tobacco products and/or legal alcoholic beverages are discovered, Security will dispose of it immediately. Spring Grove Hospital Center cannot store these products, even temporarily.
2. Medications, including herbal supplements, which are brought into the hospital by a patient or family member shall be handled in accordance with SGHC032785 (Medication Brought into the Center).
3. The Charge Nurse or designee shall document observations of the patient's response and condition when a contraband item is taken from the patient. This documentation is to be in the form of a progress note that must be entered before the end of the shift.
4. Unauthorized items that are not to be held by patients on the unit, such as personal hygiene items may be stored by nursing staff in a secure area, or,

with the patient's permission (documented in a progress note), discarded. Aerosols, lighters, and other objects that may not be safely stored on the unit may be properly disposed of by nursing staff if the patient is unable to make other arrangements for them within 24-hours. Any item that poses an immediate danger may be disposed of immediately, in consultation with Security.

5. Spring Grove Hospital Center will not compensate any person for the value of the following contraband items taken into its possession if such items are lost or destroyed: Cigarettes or other tobacco products; Alcoholic Beverages; Weapons such as guns or knives; Illegal materials or other illegal substances; and objects that cannot be safely stored at Spring Grove.

Related policies:

Contraband Items (attachment)

References:

Medication Brought into the Center Valuables

Photographing, Videotaping and/or Audiotaping Patient

Razor Use

Approved

 11/30/23
 Monica Chawla, M.D., FAPA, Date
 Chief Medical Officer

 11/30/23
 Marie Rose Alam, M.D., FAPA, Date
 Chief Executive Officer

MRA/tg

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